COUN 5553 Quiz 1 Review Packet

Chapters 1-8 | Lectures 1-4 | ACA Code of Ethics

Open Notes Quiz — Use Ctrl+F liberally

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1. KEY DEFINITIONS & TERMS

The Big Three: Legal vs. Ethical vs. Best Practice

Term	Definition	Created By
Legal standards	The *minimum* behavior society will tolerate from a professional	Elected officials (legislators); enforced by police; interpreted by judges
Ethical standards	The *minimum* behavior other counselors will tolerate from counselors	Created by counselors themselves (e.g., ACA Code of Ethics); enforced by ethics committees & licensure boards
Best practice	The *ideal* for counselor practice — what counselors aspire to	Professional consensus; aspirational, not mandatory

Values & Ethics Vocabulary

Term	Definition
Morals	From Latin *mores* (customs/norms); moral actions determined within broad cultural context or religious standard; personal belief system
Personal values	Influenced by culture, family, community, beliefs; unique to the individual; guide what is good/right behavior

Professional values	Identified values of a professional group (e.g., ACA's core values)
Ethics	A discipline within philosophy concerned with human conduct and moral decision making
Professional ethics	Conduct judged as good or right for counselors as a professional group
Principle ethics	Professionally agreed-upon obligations always considered in ethical decision making; asks "What should I do?"
Virtue ethics	Focus on traits of character/virtue; asks "Who should I be?" (not just as clinician but as a person)
Bracketing	"The intentional separating of a counselor's personal values from professional values in order to provide ethical and appropriate counseling to all clients" (Kocet & Herlihy, 2014, p. 182)

Six Principle Ethics of Counseling (ACA, 2014 Preamble)

Principle	Definition	Plain English
Autonomy	Fostering the right to control the direction of one's life	Clients choose their own direction; counselors decrease dependency
Nonmaleficence	Avoiding actions that cause harm	"Do no harm" — the most salient moral principle
Beneficence	Working for the good of individuals and society	Actively promote mental health and well-being
Justice	Treating individuals equitably, fostering fairness and equality	Fair treatment, nondiscrimination, equal access
Fidelity	Honoring commitments and keeping promises; fulfilling responsibilities of trust	Be faithful — especially to confidentiality pledges
Veracity	Dealing truthfully with individuals	Be honest in all professional contacts

Ethical Theories

Theory	Position
Ethical absolutism	Some absolute moral standards are universally applicable regardless of circumstances
Ethical relativism	No absolute moral standards; what is morally right depends on culture/context
Utilitarianism (Mill)	Choose the act that does the greatest good for the greatest number (consequential)

Deontology (Kant)	An action is right because the *principle* guiding it is right, not because of consequences (nonconsequential)
Egoism	Actions taken out of self-interest
Altruism	Actions taken to benefit others
Ethics of care / Relational ethics (Gilligan)	Ethics exists in relationships; solution found in strengthening relationships, not abstract principles
Virtue ethics (Aristotle)	Positive character developed through consistently taking actions based on society's values; focus on the whole person

Other Key Terms

Term	Definition
Fiduciary relationship	Legal term: counselor acts in the client's benefit; client is not there for counselor's benefit
Transference	Client projects past feelings/attitudes toward significant people onto their therapist
Countertransference	Counselor's own feelings, attitudes, experiences that arise in work with a client
Disclosure statement	Written contract for informed consent; discloses nature of the counseling relationship
Privileged communication	Legal concept protecting clients from having confidential communications disclosed in court without permission
Confidentiality	Explicit contract/promise not to reveal client information except under agreed circumstances
Privacy	Broadest term; individual's right to decide which personal information is shared
Malpractice	Professional misconduct or unreasonable lack of skill resulting in harm to client
Proximate cause	Reasonably close causal connection between counselor's conduct and resulting injury
Subpoena	Legal document compelling production of records or testimony
HIPAA	Health Insurance Portability and Accountability Act — federal law requiring specific written consent and privacy protections
SOAP notes	Subjective, Objective, Assessment, Plan — format for clinical notes
DAP notes	Data, Assessment, Plan — alternative note format
Slippery slope	When one begins to compromise principles, it becomes easier to continue sliding

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2. CHAPTER 1: INTRODUCTION TO ETHICS

Professional Orientation Has 3 Components

1. **Developing a professional identity** as a counselor
2. **Learning about ethics** — standards, reasoning, decision-making skills
3. **Learning about the law** as it applies to counseling

Model for Professional Practice (Figure 1-1)

"Built from within but balanced from outside the self"

Internal building blocks (bottom to top):

1. Intentionality — wanting to do the right thing
2. Moral principles & virtues
3. Knowledge of ethical, legal, and professional standards
4. Self-awareness
5. Decision-making skills and models
6. Courage of your convictions

External supports:

- Consultation with colleagues
- Supervision
- Continuing education
- Code of ethics
- Laws (e.g., privileged communication statutes)
- Agency/system policies

Key Points from Chapter 1

- Ethics prevails over personal values when conflicts arise in counseling
- The counseling relationship exists to benefit the client
- Few absolute right answers exist — dilemmas require careful reasoning
- Ethical reasoning is an *acquired skill*, not an inherent gift
- ACA Code of Ethics standards must be interpreted considering uniqueness of client, situation, and cultural variables
- The "slippery slope" phenomenon: compromising principles becomes easier over time

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3. CHAPTER 2: PROFESSIONAL IDENTITY OF COUNSELORS

Consensus Definition of Counseling (20/20 Vision, 2010)

> *"Counseling is a professional relationship that empowers diverse individuals, families, and groups to accomplish mental health, wellness, education, and career goals."*

- Accepted by 29 of 31 major counseling organizations (School Counselors and Counselors for Social Justice did not accept at the time)

Philosophy of Counseling: The Wellness Model

The counseling profession is distinct from psychology/psychiatry in its focus:

1. **Wellness model** (not illness/medical model) — assumes health

2. **Developmental perspective** — lifespan, stages, growth
3. **Prevention and early intervention** — preferred over remediation
4. **Individual empowerment** — teach clients to solve problems independently

- Terminal degree for licensure: **Master's degree** (unlike psychology which requires a doctorate)

Other Mental Health Professions (p. 58-59)

Profession	Degree	Focus
Psychiatrist	MD	Medical model, prescribing ability
Clinical Psychologist	PhD	Medical model, assessment, research, psychopathology
Clinical Social Worker	MA	Diversity, human rights, social/economic justice, policy
Professional Counselor	MA	Wellness, development, prevention, empowerment

CACREP (Council for Accreditation of Counseling and Related Educational Programs)

- Sets standards for counselor preparation
- Accredits training programs
- Helps with licensure portability
- Preps for NCE exam
- Foundation for doctoral studies
- 8 core areas all counseling students must study (including Professional Orientation and Ethical Practice, Social/Cultural Identities, Lifespan Development, etc.)

Credentialing

- **Licensure** — most powerful; established by state law; required for practice/billing
- **Certification** — voluntary, shows competence/specialty (e.g., NCC from NBCC)
- **Accreditation** — program-level (CACREP)
- Licensure reciprocity across state lines is a current issue; Counseling Compact working toward this

ACA (American Counseling Association)

- Primary professional organization: www.counseling.org
- 4 regions, 19 divisions, state branches
- Student rates available
- Professional values (ACA, 2014):
    1. Enhancing human development across the lifespan
    2. Honoring diversity
    3. Embracing a multicultural approach
    4. Promoting social justice
    5. Safeguarding the integrity of the counselor-client relationship
    6. Practicing competently and ethically

Current Issues in Counseling

- Danger of fragmenting into specialties vs. one unified profession
- Standardization of licensure requirements across states
- Scope of practice disputes with other mental health professions
- Job classifications that don't include counseling titles
- Third-party reimbursement
- Telehealth standards

- Licensure reciprocity (Counseling Compact: counselingcompact.org)

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4. CHAPTER 3: MULTICULTURALISM, SOCIAL JUSTICE, ADVOCACY & VALUES

Key Concepts

- **ACA Code requires** counselors take an active role understanding clients' diverse cultural backgrounds
- **Multicultural counseling competency** is required across ALL counseling specialties (C.2.a.)
- Counselors wield power over clients; avoiding abuse of that power in cultural differences is an ethical responsibility

Cultural Competency Components

1. **Attitudes and Beliefs** — awareness of social identities, biases, privileges
2. **Knowledge** — decrease assumptions, increase understanding of worldviews
3. **Skills** — reflective abilities, critical thinking, broaching, curiosity
4. **Actions** — advocating for change at individual and community levels

ADDRESSING Model (Hays, 2002)

- **A**ge
- **D**evelopmental and acquired **D**isability
- **R**eligion/spirituality
- **E**thnicity
- **S**ocioeconomic status
- **S**exual orientation
- **I**ndigenous status
- **N**ationality
- **G**ender

RESPECTFUL Model (D'Andrea & Daniels, 2001)

More psychological/individual components: Religious/spiritual ID, Ethnic ID, Sexual ID, Psychological ID, Economic class, Chronological challenges, Threats to wellbeing, Family history, Unique physical challenges, Location of residence

Intersectionality (Crenshaw, 1989)

Compounded oppression for those with multiple minority identities

Social Justice vs. Advocacy

- **Social justice** — promoting access for all people to mental health services, especially marginalized
- **Advocacy** — commitment to social justice through *action* to promote change

Cultural Humility

- Recognizing our limitations
- Willingness to: seek consultation, seek continuing education, make referrals
- The most important resource for learning about cultural differences is **YOUR CLIENT**

Values and Referrals

- **Appropriate referral reasons** (A.11.a, c): client no longer needs services, unlikely to benefit, at risk of harm, doesn't pay fees, counselor lacks competency, counselor in jeopardy
- **INAPPROPRIATE referral reasons** (A.11.b, d): discrimination, differences in values/attitudes/beliefs/personality, abandonment situations
- **A.11.b**: "Counselors refrain from referring prospective and current clients based solely on the counselor's personally held values"
- Professional counselors may NOT deny services to LGBTQ+ individuals (or any protected class) based on counselor's values
- Referrals must be based on **skill-based competency**, not values

Bracketing in Practice

- Must know your own values to bracket them
- Without bracketing: emotional suppression, denial, or fantasy may occur (Sells & Hagadorn, 2016)
- James Marcia's Theory: identities should be defined internally, not externally (foreclosure)

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5. CHAPTER 4: CLIENT WELFARE & INFORMED CONSENT

Client Welfare

- Counselors are responsible for safeguarding clients' welfare from initiation through termination
- Counselors have a **fiduciary relationship** with clients (legal term: you act in their benefit)
- Must be aware of personal needs/motivations and countertransference reactions

Countertransference Warning Signs

- Being overprotective, treating clients in overly kind ways
- Rejecting a client
- Needing constant reinforcement/approval
- Seeing yourself in clients
- Developing sexual/romantic feelings
- Giving advice compulsively
- Desiring social relationships with clients

Mandated Clients (ACA A.2.e)

- May refuse services; counselor must explain consequences of refusal
- Must be accurate, honest, objective in evaluations (C.6.b)
- Court mandates often require disclosure to court
- Make clear if purpose is evaluation vs. counseling
- View counseling and evaluation as **two separate processes** (ideally two different clinicians)
- Written informed consent required to move from therapeutic to evaluative role

Treatment Plans & Techniques

- Goals and treatment plans established **jointly** with clients
- Techniques grounded in **empirical research** (C.7.a)
- Experimental techniques require discussion and possibly written disclosure
- Potential risks and benefits must be disclosed

Termination Reasons

- Client reached goals
- Client not benefiting or being harmed
- Client doesn't pay fees

- Counselor in jeopardy of harm
- Counselor or client moves
- Have a plan for unexpected termination

Avoiding Abandonment (ACA A.12)

- Do NOT terminate when client is in crisis or angry
- Honestly discuss intention to terminate with specific reason
- Give client opportunity to make changes
- Give written notice
- Offer 1-3 termination sessions
- Give up to 3 good referrals
- Give emergency contact info
- Document in client file
- Give plenty of time to find new provider
- Transfer records promptly

Informed Consent

- Both a **legal** and **ethical** obligation
- Demonstrates respect for autonomy and self-determination
- Integral to forming effective therapeutic alliance
- Written contracts = **disclosure statements**
- Ongoing process, both oral and written
- HIPAA and state laws require specific written consent prior to services

10 Elements of Informed Consent

1. Purposes, goals, techniques, procedures, limitations, potential risks and benefits
2. Counselor's qualifications
3. Information about termination and referrals
4. Emergency protocols and crisis resources
5. Role of technology (and backup plan if tech fails)
6. Implications of diagnosis and intended use of testing/reports
7. Fees and billing information
8. Confidentiality and its limitations
9. Right to obtain records and actively participate in counseling plans (see HIPAA requirements)
10. Right to refuse recommended services or modality change and be advised of consequences

8 Areas That Could Lead to Legal Problems (p. 100)

1. Failing to include required HIPAA elements
2. Guaranteeing an outcome
3. Guaranteeing privacy with no exceptions
4. Agreeing to unintended fee arrangements
5. Touching without permission
6. Misrepresenting credentials
7. Failing to communicate the nature of counseling
8. Neglecting to warn about possible stigma (of diagnosis)

Diversity Considerations

- Informed consent as traditionally conceptualized may not be respectful of all cultures
- Use language clients can understand
- Consider cultural implications and adjust practices accordingly

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6. CHAPTER 5: CONFIDENTIALITY & PRIVILEGED COMMUNICATION

Three Related but Distinct Concepts

Concept	Type	Definition
Privacy	Broadest	Client's right to decide which information is shared
Confidentiality	Ethical	Explicit promise not to reveal client info except under agreed circumstances
Privileged communication	Legal	Protects clients from having communications disclosed in court without permission

Confidentiality

- "One of the most fundamental of all professional obligations in counseling"
- Cornerstone on which trust is built
- Based on principle of **fidelity** (keeping promises)
- Entire **Section B** of ACA Code is devoted to it (the only issue with its own section)
- Confidentiality cannot be guaranteed when more than one client is in the room (groups, family therapy)

Limits to Confidentiality (Exceptions)

1. **Client signs a Release of Information (ROI)**
2. **Duty to warn** — client poses danger to identifiable others (including life-threatening diseases, B.2.c)
3. **Duty to protect** — client is danger to self (suicidal) or others
4. **Duty to report** — suspected abuse/neglect of children or vulnerable adults
5. **Involuntary hospitalization** proceedings
6. **Client brings lawsuit** against counselor
7. **Subpoena** from secretary of health in response to complaint
8. **Court-ordered disclosure** (B.2.d — counselor should still seek client consent or limit disclosure)

Privileged Communication

- **Legal concept** (not ethical)
- Privilege belongs to the **CLIENT**, not the counselor
- Client must assert the privilege
- If client cannot be located, counselor asserts privilege on their behalf
- If client is deceased, legal representative asserts privilege

WA State Relationships That Enjoy Privilege (RCW 5.60.060)

- Doctor/Patient
- Lawyer/Client
- Priest/Penitent
- Spouse/Spouse
- LMHC, LMFT, LCSW/Client
- Sexual Assault or DV Advocate
- AA Sponsor
- Psychologist/Patient

- **During practicum/internship, counseling students have the same ethical obligations to uphold confidentiality as licensed counselors**

9 Exceptions to Privileged Communication (Glosoff et al., 2000)

1. Complaints filed after counselor-client dispute
2. Client litigating their mental condition
3. Client danger to self or others
4. Report of child abuse
5. Client contemplating serious crime
6. Court-ordered psychological examination
7. Involuntary hospitalization proceedings
8. Client was victim of a crime
9. Client accused of harming a vulnerable adult

Sharing Information

- Administrative staff handling confidential info: privilege extends to them
- Supervision: include in informed consent that you're getting supervision (ACA F.1.c); client identity usually shared
- Consultation: make every effort to protect client identity; get consent if cannot
- Referrals/past records: get client consent

When More Than One Person Is Present

- Confidentiality cannot be guaranteed
- Family system or couple becomes identified client (unless otherwise established in informed consent)
- Discuss limits in informed consent before counseling begins

Children Ages 13-18 in WA State

- **RCW 71.34.530**: Any minor 13+ may request and receive outpatient treatment without parental consent
- **RCW 70.02.130**: If minor is authorized to consent, only the minor may exercise patient rights regarding that care

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7. CHAPTER 6: RECORDS & SUBPOENAS

ACA on Records (B.6.a-h)

Standard	Requirement
B.6.a	Maintain records necessary for professional services
B.6.b	Records must be private/secure
B.6.c	Must have client permission to record sessions
B.6.d	Must have permission to observe/review sessions
B.6.e	Competent clients should have reasonable access to records
B.6.f	Provide assistance interpreting records
B.6.g	Written permission to disclose/transfer to third parties

B.6.h	Securely store records after termination; **5 years in WA** (WAC 246-809-035); be cautious about destroying records that may be needed in court

Why Keep Good Notes?

- Track goals, progress, change, turning points
- Document important actions/decisions (protects counselors if challenged)
- Remember where sessions left off
- Continuity of care if client changes counselors
- Write with understanding someone may read them later
- Clear enough for another professional to follow

SOAP Notes

- **S**ubjective — client reported (perceptions, feelings, experiences)
- **O**bjective — counselor observed (hand-wringing, lack of eye contact, appearance)
- **A**ssessment — making sense of S and O
- **P**lan — action steps toward treatment goals

DAP Notes

- **D**ata
- **A**ssessment
- **P**lan

HIPAA Requirements for Counselors

- Give clients clear written explanation of how you use, keep, and disclose their health care information
- Give clients access to their records (unless it would cause them harm)
- Follow a written process for clients to request amendments
- Keep written history of disclosures, available to clients
- HIPAA exception: may share with family/others assisting in care if client informed and given opportunity to object

If Client Requests Records

1. Document and get signed release
2. Explain risks of disclosure
3. Ensure records would not harm client
4. Ensure other clients' info not disclosed
5. Provide assistance reviewing records
6. Have clear written policy

Subpoenas

- Used to: obtain records, compel testimony, require written responses, require appearance with records
- **If subpoenaed**: Don't ignore but get advice before responding; contact client; talk to supervisor/attorney; do NOT just hand over records; check if information is privileged; get ROI from client (B.2.d)

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8. CHAPTER 7: COMPETENCE, ASSESSMENT & DIAGNOSIS

What Is Counselor Competency?

- Complex construct on a continuum
- Ethical: "do no harm" (nonmaleficence is most salient)
- Skill: "perform at an acceptable level"
- Practice: "within the boundaries of competence"
- Offer ethical referrals when outside scope (not for discriminatory reasons)

How Competence Is Established

1. **Graduate programs** — admissions, curriculum, supervised experience, gatekeeping, remediation
2. **Licensure** — most powerful credentialing; established by state law
3. **Certification** — voluntary evidence of competency/specialty
4. **Continuing education** — required by most licensure boards (ACA C.2.a)

WA State Licensure

- **LMHC-A**: education requirements + application
- **LMHC**: education + application + passing NCE or NCMHCE + 3000 hours (1200 direct, 100 supervision) or 3 years full-time

Maintaining Competence (4 methods)

1. **Continuing education** — required by most boards
2. **Peer review/consultation** — lifelong process; enhanced self-awareness, support, feedback
3. **Technology** — internet resources for staying current
4. **Making referrals** — when client exceeds your competence (based on competence, NOT discrimination)

Erosion of Competence: Burnout & Impairment

- **Burnout**: decline in self-care, strained relationships, damaged emotional health
- **Impairment**: unable to perform responsibilities, putting clients at risk
  - Often associated with substance abuse and blurring of boundaries
  - Risk factors: environmental factors, vicarious traumatization, preexisting conditions
- **ACA C.2.g and F.5.b**: counselors must monitor themselves; if impaired, must limit/suspend/terminate professional responsibilities and seek assistance

Assessment & Diagnosis

- Counselors have education/skills for evaluations
- Clients being assessed need to understand: process differs from counseling, what it entails, what the product will be, how it will be used
- Written informed consent required before forensic evaluations
- Expert witness: avoid unless agreed to; if required by job, be prepared to testify

Testing

- Competence depends on training/experience with the specific test
- Clients have right to informed choice about participation
- Clients can expect explanation of results
- **Diversity issues**: consider validity, reliability, norming populations; place results in proper perspective with cultural factors

Diagnosis

- Usually using DSM-5-TR (medical model)

- Often required for insurance reimbursement
- Clients must understand implications of any diagnosis
- Diversity considerations: culture affects how problems are defined and experienced (E.5.b); be aware of historical prejudices in misdiagnosis (E.5.c)

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9. CHAPTER 8: MALPRACTICE & RESOLVING LEGAL/ETHICAL CHALLENGES

Malpractice Definition

Professional misconduct or unreasonable lack of skill; failure to render services to the expected level, resulting in harm.

4 Elements Client Must Prove for Malpractice

1. Counselor had a **duty** to use reasonable care
2. Counselor **failed** to conform to required duty of care (breach)
3. Client was **injured**
4. **Proximate cause** — reasonably close causal connection between counselor's conduct and injury
5. Client suffered **actual loss** or damage

Most Common Reasons for Malpractice Lawsuits

1. **Sexual relationships with clients** (biggest payouts)
    - WA state law (WAC 246-16-100): counselor may NEVER engage in sexual relationship with client OR former client — more restrictive than ACA (which has 5-year rule)
2. **Client suicide** (second leading cause)

Clients at Risk for Suicide

- Confidentiality waived when disclosure necessary to protect from serious/foreseeable harm (B.2.a)
- First responsibility: determine if client may be in danger
- Legal duty to accurately evaluate suicide potential
- Can be held liable for overreacting AND underreacting
- Non-suicidal self-injury (NSSI) is separate from suicidal ideation — assess separately

Suicide Assessment Questions

- Do you have a plan?
- Can you tell me specifically what you plan to do?
- Do you have access to the means?
- Have you ever attempted before? What happened?

Guidelines for Assessing Suicidal Behavior

- Take direct verbal warnings seriously
- Pay attention to previous attempts
- Identify depression
- Be alert for hopelessness/helplessness
- Monitor severe anxiety/panic attacks
- Determine if they have a plan
- History of substance abuse
- Behavioral changes (giving away possessions, finalizing affairs)
- History of psychiatric treatment

Clients at Risk for Harming Others

- ACA B.2.a: confidentiality does not apply when disclosure required to protect from serious/foreseeable harm
- Range of actions: least intrusive (promise not to harm) to most intrusive (involuntary commitment)
- Options: notify family, persuade voluntary commitment, notify police, call periodically

If Formal Ethics Complaint Filed Against You

- Respond fully and in writing to all charges
- Consult with an attorney
- Submit documentation
- Contact professional liability insurance carrier
- **Do NOT contact the client** who filed the complaint

If Angry Client Threatens to Sue

- Respond directly if possible
- Calm the person; listen to concerns
- Do not admit to wrongdoing
- Don't say anything you wouldn't want repeated under oath

Guidelines for Preventing Problems

- Document everything
- Maintain good records
- Consult regularly
- Stay current with ethical standards
- Carry professional liability insurance
- Follow informed consent procedures

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10. KEY LEGAL CASES

Tarasoff v. Regents of University of California (1976)

- **Established**: Duty to warn and duty to protect
- **Facts**: Poddar told a doctor of his obsession and desire to kill Tatiana Tarasoff. Police were called; Poddar said he wouldn't act. He then killed Tarasoff.
- **Ruling**: Therapists must take reasonable steps to protect intended victims. May include warning victim, notifying police, or other necessary steps.
- **Key principle**: "Therapist must take reasonable steps to care and protect the intended victim"

Volk v. DeMeerleer (WA State)

- **Expanded Tarasoff** in Washington State
- **Facts**: DeMeerleer (history of severe mental illness, rageful thoughts) murdered his girlfriend Rebecca. Psychiatrist Ashby reported SI but no plan to act, assessed stability. DeMeerleer never identified a specific victim.
- **Ruling**: "A therapist's duty is NOT limited to a duty to warn in WA. The duty to protect can arise even if the client does NOT express an intention to harm a reasonably identifiable victim."
- **Key principle**: Broader duty to protect in WA than Tarasoff requires

Other Important Cases (from Lecture 3 slides)

Case	Established

Bradley Case	Duty not to negligently release a dangerous client
Jablonski Case	Duty to commit a dangerous individual
Hedlund Case	Extends duty to warn to anyone near the intended victim who might also be in danger
Jaffee Case	Communications between licensed psychotherapists and clients are privileged and protected from forced disclosure in federal cases

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11. ACA CODE OF ETHICS QUICK REFERENCE

Structure

- Preamble + 8 Sections (A-I):
  - **A**: The Counseling Relationship
  - **B**: Confidentiality and Privacy
  - **C**: Professional Responsibility
  - **D**: Relationships with Other Professionals
  - **E**: Evaluation, Assessment, and Interpretation
  - **F**: Supervision, Training, and Teaching
  - **G**: Research and Publication
  - **H**: Distance Counseling, Technology, and Social Media
  - **I**: Resolving Ethical Issues

6 Purposes of the ACA Code

1. Set forth ethical obligations and guidance
2. Identify ethical considerations for counselors and trainees
3. Clarify ethical responsibilities held in common
4. Serve as guide for constructing best course of action
5. Support the mission of ACA
6. Serve as basis for processing ethics complaints

Most Testable ACA Standards by Section

Section A: The Counseling Relationship

Standard	Key Content
A.1.a	Primary responsibility: respect dignity and promote welfare of clients
A.1.c	Counseling plans devised jointly; regularly reviewed
A.1.d	Consider support networks with client consent
A.2.a	Informed consent: ongoing, documented, both written and verbal
A.2.b	Types of information needed (purposes, goals, techniques, fees, confidentiality limits, etc.)
A.2.c	Developmental and cultural sensitivity in communication

A.2.d	Minors/incapacitated adults: seek assent; balance rights
A.2.e	Mandated clients: discuss limitations, explain info sharing, client may refuse
A.4.a	Avoiding harm
A.4.b	Personal values: avoid imposing; respect diversity; seek training
A.5.a	Sexual/romantic relationships with current clients PROHIBITED
A.5.b	Cannot counsel someone you previously had sexual relationship with
A.5.c	Former clients: prohibited for 5 years; then must document no exploitation/harm
A.6.b	Extending boundaries: consider risks/benefits; document rationale
A.8	Multiple clients: clarify who is the client
A.10.e	Bartering: only if not exploitive, client requests it, accepted practice
A.10.f	Gifts: consider cultural context, therapeutic relationship, value, motivations
A.11.a	Competence within termination/referral
A.11.b	Values within termination/referral — do NOT refer based solely on personal values
A.11.c	Appropriate termination — when client no longer needs help, not benefiting, or being harmed
A.12	Abandonment: do not abandon or neglect; make arrangements for continuation

Section B: Confidentiality and Privacy

Standard	Key Content
B.1.a	Multicultural considerations in confidentiality
B.1.b	Respect for privacy; request private info only when beneficial
B.1.c	Protect confidential info; disclose only with consent or legal/ethical justification
B.1.d	Explain limitations of confidentiality at initiation and throughout
B.2.a	Serious and foreseeable harm exception; legal requirements
B.2.b	End-of-life decisions: option to maintain confidentiality
B.2.c	Contagious, life-threatening diseases: may disclose to identifiable third parties at risk
B.2.d	Court-ordered disclosure: seek consent or limit disclosure
B.2.e	Minimal disclosure: only essential information
B.3.a	Subordinates must maintain privacy
B.4.a	Group work: explain confidentiality parameters
B.4.b	Couples/family: define who is "the client"; default = couple/family

B.5.a-c	Clients lacking capacity: protect confidentiality per law; inform parents; seek permission for release
B.6.a-h	Records: maintain, keep secure, get permission to record/observe, client access, written permission for transfer, store securely after termination
B.7.a-b	Consultation: protect identity; disclose only with consent or necessity

Section C: Professional Responsibility

Standard	Key Content
C.1	Know and follow ACA Code and applicable laws
C.2.a	Practice within boundaries of competence; multicultural competency required
C.2.b	New specialty areas: get training first
C.2.d	Monitor effectiveness
C.2.g	Impairment: monitor self; limit/suspend/terminate if impaired
C.5	Nondiscrimination
C.6.b	Accurate, honest, objective evaluations (relevant to mandated clients)
C.7.a	Techniques grounded in empirical research

Section E: Evaluation, Assessment, Interpretation

Standard	Key Content
E.5.b	Culture affects how problems are defined; consider socioeconomic/cultural experience in diagnosis
E.5.c	Be aware of historical prejudice in misdiagnosis
E.6.a	Consider validity, reliability, limitations of instruments; use multiple forms of assessment
E.8	Use caution with assessments normed on different populations; consider cultural factors

Section F: Supervision

Standard	Key Content
F.1.c	Include supervision in informed consent
F.2.b	Address multiculturalism in supervisory relationship
F.5.b	Supervisees monitor impairment in themselves

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12. 8-STEP ETHICAL DECISION-MAKING MODEL

(From text p. 19-21 and Lectures 1 & 2)

Keep in mind: **cultural context** and **your emotions** throughout.

1. **Identify the problem or dilemma** — Is it a legal issue or a clinical issue? Take time to review carefully.
2. **Involve your client** in the decision-making process — empowers the client, builds the relationship, safeguards integrity.
3. **Review relevant ethics codes** — review codes, laws, and literature.
4. **Consider the principles and virtues** — not just what to DO but who you want to BE.
5. **Tune into your feelings** — aware of emotions, beliefs, values, motivations. Body relaxed, mind engaged.
6. **CONSULT with colleagues or experts** — important from a legal standpoint too.
7. **Consider the context** — be culturally competent; consider ramifications for client's family, community, other professionals.
8. **Make a decision and act** — don't linger in indecision; have courage.

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13. WASHINGTON STATE SPECIFIC LAWS

Licensure

- **LMHC-A**: education requirements + application
- **LMHC**: education + NCE/NCMHCE + 3000 hours (1200 direct, 100 supervision) or 3 years full-time
- 50 hours postgraduate supervision for CACREP grads

Privileged Communication (RCW 5.60.060)

- Extends to: Doctor/Patient, Lawyer/Client, Priest/Penitent, Spouse/Spouse, LMHC/LMFT/LCSW/Client, DV/SA Advocates, AA Sponsors, Psychologist/Patient

Sexual Relationships with Clients (WAC 246-16-100)

- **NEVER** — with current OR former clients
- **More restrictive** than ACA (which allows after 5 years with documentation)

Records Retention (WAC 246-809-035)

- **5 years** after termination in WA

Minors and Consent

- **Outpatient mental health**: age 13+ can consent without parental consent (RCW 71.34.530)
- **Outpatient substance abuse**: age 13+
- **STD testing/treatment**: age 14+
- **Birth control**: any age
- **Sexual activity age of consent**: 16 (Chapter 9A.44 RCW)
    - 14-15: partner must be less than 4 years older
    - 12-13: partner must be less than 3 years older
    - Under 12: partner must be less than 2 years older

Family-Initiated Treatment (for kids 13+)

- **Outpatient**: parent may seek services for up to 12 sessions in 3 months without adolescent consent; then adolescent must consent (RCW 71.34.650)
- **Inpatient**: adolescent may give written notice to leave at any time

Child Abuse & Neglect (RCW 26.44)

- **Definition** (RCW 26-44-020): injury, sexual abuse, sexual exploitation, negligent treatment, maltreatment indicating child's health/welfare/safety is harmed

- Physical discipline is NOT abuse when reasonable and moderate by parent/teacher/guardian (RCW 9A.16.100)
- **Presumed unreasonable**: throwing, kicking, burning, cutting; striking with closed fist; shaking child under 3; interfering with breathing; threatening with deadly weapon; causing bodily harm greater than transient pain
- **Mandatory reporters** must report (RCW 26.44)
- Report to: 1-866-END HARM (1-866-363-4276) or county numbers
- King County: 1-800-609-8764

Vulnerable Adults (WA Definition)

- Person 60+ lacking functional/physical/mental ability to care for self
- Adult with developmental disability
- Adult with legal guardian
- Adult in long-term care facility
- Adult receiving home services

Death with Dignity Act (RCW 70.245)

- Passed November 4, 2008
- Clients with less than 6 months to live may request lethal medication
- ACA allows counselors option of confidentiality
- Seek supervision and legal advice

Ricky's Law

- Procedures for reporting individuals at risk of suicide/self-harm
- Be familiar with your site's procedures

Duty to Warn/Protect in WA

- **Broader than Tarasoff** per *Volk v. DeMeerleer*
- Duty to protect can arise even without identified victim or expressed homicidal ideation
- Therapist must act with "reasonable care, consistent with the standards of the mental health profession"

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14. CHRISTIAN INTEGRATION (WATSON, 2018)

Character Virtues Mapped to Professional Ethics & Spiritual Disciplines

Character Virtue	Professional Ethic	Spiritual Disciplines
Faith	Fidelity	Contemplative prayer & church involvement
Hope	Beneficence	Sabbath & worship
Love	Respect/Autonomy	Intercessory prayer
Justice	Justice	Attentiveness, hospitality & service
Temperance	Nonmaleficence	Examen & fasting
Courage	Integrity/Veracity	Rule of life & example
Practical Wisdom	Competence	Scripture & discernment

Faith and Fidelity (Watson, 2018)

- Aquinas: faith involves "believing someone and believing something"
- Watson argues faith connects to trust; therapists live out faith through building trust in relationships
- Fidelity = same root as fiduciary: "honoring commitments and keeping promises"
- Cultivating faith: contemplative prayer and church involvement
- "We uphold the ethical principle of fidelity not only because we have to but because it has become an essential part of our personal and professional character" (Watson, 2018, p. 46-47)

Justice (Watson, 2018; Aquinas)

- Aquinas: "to render to each person that which is his or her right"
- Other-oriented virtue (vs. other cardinal virtues that are individually oriented)
- ACA focus: prosocial behavior, equal access, treating individuals equitably
- Christian doctrine: all persons created by God with eternal value
- Key scriptures: Micah 6:8 ("act justly, love mercy, walk humbly"), Isaiah 1:17, Psalm 106:3, Isaiah 61:1/Luke 4

Wisdom and Discernment

- Dallas Willard: study as spiritual discipline — "focusing the mind upon God's works and words"
- Ignatius of Loyola: discernment as a habit — "finding God in all things"
- Ruth Haley Barton: prayerful discernment includes collecting data, inner reflection, action, submission to God's will

Developing Virtue (Neuroscience Findings)

1. Developing ingrained habits of moral thought/action over time (automatic cognitive processes)
2. Moral exemplars facilitate moral thinking (mirror neuron research)
3. Religious communities are effective contexts for modeling virtuous habits

Exclusion and Embrace (Volf, 2019)

- "Double vision" — seeing through another's perspective empathically without leaving behind own views
- High level of differentiation between self and others required
- Paul in Romans 3:23: "all have fallen short" — understanding our part in cultural sin

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QUICK-HIT STUDY TIPS FOR THE QUIZ

Since this is **open notes**, organize for speed:

1. **Use Ctrl+F** to search this document for keywords
2. The most commonly tested areas (per syllabus) are:
    - ACA ethical standards and their application
    - Distinguishing legal vs. ethical vs. professional/best practice
    - The 6 principle ethics (know definitions cold)
    - Informed consent elements
    - Confidentiality exceptions (duty to warn/protect/report)
    - Legal cases (especially Tarasoff and Volk)
    - WA state-specific laws (minors, privileged communication, sexual relationships)
    - Malpractice elements
    - Ethical decision-making model (8 steps)
    - Referral: appropriate vs. inappropriate reasons

    - Competence and impairment
3. **Know your ACA section letters**: A = Counseling Relationship, B = Confidentiality, C = Professional Responsibility
4. **Tricky distinctions**:
    - Privacy vs. Confidentiality vs. Privileged Communication
    - Principle Ethics ("What should I do?") vs. Virtue Ethics ("Who should I be?")
    - Legal standards (minimum/society) vs. Ethical standards (minimum/profession) vs. Best Practice (ideal)
    - WA law on sex with former clients (NEVER) vs. ACA (5-year prohibition)
    - Tarasoff (identified victim) vs. Volk (no identified victim needed in WA)